



OFFICIAL

NMHS MHS PROCEDURE

Alcohol, Illicit Substances and Non-Prescribed Substances Management of

Scope (Audience)	All NMHS Mental Health Services (MHS) Inpatient Staff
Scope (Area)	All NMHS MHS Inpatient Areas – Graylands Hospital, Sir Charles Gairdner Hospital, Osborne Lodge, Selby Lodge and State Forensic Mental Health Service (SFMHS)
Summary	Procedure provides direction to staff to remove, store / confiscate and record prohibited items legally and safely when encountered at North Metropolitan Health Service Mental Health (NMHS MH) inpatient units
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2) Comprehensive Care (Std 5) Recognising and Responding to Acute Deterioration (Std 8)
Policy Sponsor	Inpatient Co Directors MHSDHS
Policy Contact	Operational Inpatient Co Director MHSDHS
Version	4.0
Effective Date	20 November 2025
Review Date	20 November 2028

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

The procedure will provide direction to staff to remove, store/confiscate and record prohibited items legally and safely when encountered at NMHS Mental Health Services (NMHS MHS) inpatient units.

2. Background

If a patient is suspected of possessing a dangerous or illegal object or substance that poses a risk of harm to the patient and/or other people, the health care provider may take appropriate precautions to guard against the risk of harm. This procedure should be read in conjunction with the [MHS Search of a Person, Property, Environment and Seizure Procedure](#).

This document describes the procedures that must be implemented by NMHS MHS staff if there is reasonable concern/suspicion that an inpatient has consumed or is in possession of alcohol and/or illicit or harmful substances. If staff suspect a patient in their care or a visitor, is illegally in possession of, using or supplying what is suspected of being an illicit substance as defined by the [Misuse of Drugs Act \(1981\)](#), Security are to be notified immediately and the matter is reported to West Australian Police Force (WAPOL).

3. Risk

Patients have a right to receive care in a safe environment, free of substance use. If alcohol, illicit substances or non-prescribed drugs are permitted into an inpatient unit, safety cannot be adhered to.

The use of alcohol, illicit substances, or non-prescribed drugs by patients in mental health inpatient services may exacerbate symptoms, trigger relapse into acute phases of illness and lead to self-harm or violence to others.

NMHS MHS are committed to providing a safe and secure environment for all patients, visitors, and staff at all of its sites and services.

NMHS MHS provide care and treatment for individuals using alcohol, illicit substances and non-prescribed drugs and has responsibility for preventing others from exposure to these substances while an inpatient of the service.

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Key Points

- Information regarding the use or possession of alcohol, illicit substances or non-prescribed drugs not being allowed within an inpatient unit must appear in the Patient Information brochures/booklets supplied to patients on admission.

- Signage must be displayed at the entrance to the inpatient unit advising patients and visitors that alcohol, illicit substances or non-prescribed drugs are prohibited on inpatient units.
- All patients are to be informed on admission that the use of alcohol, illicit substances or non-prescribed drugs will not be permitted whilst they are in an inpatient unit.
- All patients are to be informed on admission that as part of routine care and treatment patients may be asked about any alcohol or other psychoactive substance use.
- All patients are to be advised that during admission, or on return from leave, they may be requested to provide a urine sample for drug testing and breathe into an approved alcohol measuring device for blood alcohol measurement.
- Any staff member who becomes aware of a patient who may be in possession of/ or may have consumed alcohol, illicit substances or non-prescribed drugs is responsible for ensuring that the patient is placed on clinical observations consistent with the [NMHS MH Clinical Observation Procedure](#), for their own safety and the safety of others pending review by the Duty Doctor.
- If a patient is assessed to be medically compromised, it may be necessary to arrange transfer to an Emergency Department for further assessment and treatment.
- If a patient is in possession of a substance suspected to be illicit drugs, the substance should be secured by staff and handed over to Security who will notify WAPOL and arrange for disposal of the substance.

If at any time there is perceived danger, staff should always call a Code Black, however staff may request assistance from Security if no danger is perceived in an attempt to engage with the patient or visitor in accordance with the requirements of the [MHS Search of Person, Property, Environment and Seizure of Property Procedure](#).

5. Staff Roles and Responsibilities

- Staff must inform the Shift Coordinator immediately of the presence/consumption of alcohol, illicit substances or non-prescribed drugs, and the physical status of the patient e.g. if a patient appears sedated, intoxicated or otherwise under the influence of a potentially harmful substance.
- The Shift Coordinator must inform the Senior Registered Nurse (SRN) on duty, the responsible Medical Officer or the Duty Medical Officer.
- Vital signs, urine drug testing and alcohol measurement, safety and risk assessment are undertaken and documented in the patient's medical record.
- The patient is to be placed under clinical observation as determined by the SRN until a medical review is undertaken.
- If there is reasonable concern that a patient may be in possession of items considered a danger to themselves or others, a search of that patient's person and belongings may be undertaken in accordance with the requirements of the [MHS Search of Person, Property, Environment and Seizure of Property Procedure](#).

6. Procedures

6.1 Patients suspected of being in possession of Illicit Substances

- During the inpatient admission if concerns are identified about harmful substances, alcohol, illicit substances or non-prescribed drugs use, patients are referred to a specialist Alcohol Tobacco and Other Drugs (ATOD) Nurse and [Aboriginal Health Liaison Services](#) (if appropriate), who will provide information and brief intervention around ATOD use with supports and services that are available post discharge. Where appropriate patient's

identified support person will be offered information/psychoeducation sessions regarding ATOD supports and services.

- The Shift Coordinator/Clinical Nurse Supervisor (CNS) must inform the Security Officer and request their attendance. On arrival of Security Officer/s, staff are to request the patient to surrender the suspected illicit substance. In the event the patient refuses, nursing staff will maintain continuous visual observation until a search of the patient is conducted in accordance with the [MHS Search of Person, Property, Environment, and Seizure of Property Procedure](#).
- If staff are unsure whether a substance handed in by/found on a patient may be an illicit drug, the CNS or shift coordinator should contact the police for advice.
- If confirmed to be an illicit substance the CNS or Shift Coordinator will contact Security Officer and the police to report the incident and request the police to attend to collect the illegal substance/s.
- Prohibited substances are not to be removed from original container (i.e. drug bag or tablet container), prior to placing into secure tamper proof evidence bag.
- The following information must be recorded by clinical staff and/or Security Officers:
 - tamper proof evidence bag identification number;
 - date and time found;
 - item found/identified (description of item);
 - location of item found (i.e., left hand hip pocket);
 - location of patient when found (i.e., hospital and ward);
 - name and position of person who found item; and
 - name and position of the CNS.
- Two (2) x Staff members should sign the tamper proof evidence bag.
- Staff must **not** dispose of the substance themselves; any suspicious substance or known illegal substance should be collected by the attending police.
- The attending police will review and decide to remove the suspected substance and will issue a receipt for the confiscated substance.
- Suspicious substances or known illegal substances should **not** be kept or stored on the ward.
- The incident, observations and actions are to be documented in the patient's medical record in the chronological order of events.
- If not required by the police, the disposal of any drug taking paraphernalia found should be carried out on site. This paraphernalia should be treated as clinical waste, and sharps should be disposed of in accordance with Infection Prevention & Control policy requirements.
- Every situation will be managed on an individual basis with medical review requiring assessment of the patient's mental state, a comprehensive risk assessment, physical health assessment and identification of a management plan that includes consideration of a safe and therapeutic environment, individual risk factors and clinical observation and ongoing monitoring requirements.
- The patient's treating Consultant Psychiatrist will discuss the incident with the patient and their identified support person (if appropriate) to identify any risks as a result of the incident including suitability of the environment and ongoing treatment options.

6.2 Support of Patients post discharge

- All patients must have a comprehensive discharge plan completed and outcomes of risk assessments, interventions, management plan and follow up arrangements are documented in the patient's medical record which includes any relevant ATOD use.

- The discharge plan will be made in collaboration with the patient and their nominated support person.
- A Discharge Summary is communicated to the GP that includes referral to a specialist alcohol and other drug service as discussed with the patient.

7. Monitoring and Evaluation

CNS or delegate to complete Datix Clinical Incident Form (CIMS) when alcohol or illicit substances are detected either on a patient or in their possession or provided during a visit.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed every three (3) years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

8. Definitions

Terms	Definitions
Confiscate	The term “confiscate” refers to the removal of prohibited items from the person or their belongings, which are to be placed in the dedicated and approved secure storage for WA Police collection, and not to be returned to the person by staff.
Prohibited items	For the purpose of this policy document, “Prohibited items” include items prohibited by law and items that have the potential to place patients, visitors, and staff at risk of danger and/or harm.

Refer to [NMHS Policy Document Glossary of Terms](#) for any other frequently used terms.

9. Related Documents

Legislation	• Misuse of Drugs Act (1981)
NMHS MH Policy documents	• NMHS MH Clinical Observation Procedure • NMHS MH Clinical Risk Assessment and Management Policy • NMHS MHS Search of Person, Property, Environment and Seizure of Property Procedure • NMHS MH Person of Concern Procedure
Standards	• NSQHS Standards

10. Resources

- [NMHS MH Alcohol and Other Drugs Services](#)
- [Drug and Alcohol Clinical Advice and Support \(DACAS\)](#)
- [Community Alcohol and Drug Services \(CADS\) – Referral Form](#)
- [Mental Health Commission Next Step Drug and Alcohol Services](#)
- [Mental Health Commission – A brief guide to alcohol and drug treatment](#)
- [The “Green Book” – Directory of AOD Services in WA](#)
- [A-Z :Guide of Illicit and Licit Drugs](#)

11. Document Control

Document Version Control			
Date	Version	Author	Notes
14/06/2019	2.0	Clinical Nurse Specialist, SCGH MHU	Scheduled Review: Updated template, hyperlinks and references. Policy reviewed and amended to meet current standards and legislative requirements
06/02/2024	3.0	Director of Nursing	Scheduled review of processes, staff responsibilities, search procedures, inclusion of use of tamper proof bags, template and hyperlinks updated
20/11/2025	4.0	Inpatient Operation Co Director Adult MHS DHS	Review of processes to align with rescindment of overarching NMHS Prohibited Items Policy, SFMHS added within scope of procedure, template and resources updated

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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